

Comments Submitted in Response to RIN 0955-AA13

Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Submitted by: Miami AI Club (MAIC)

Date: [February 20th 2026]

Executive Summary

Miami AI Club (MAIC) submits these comments to support HHS's goal of accelerating safe, effective, and trusted AI adoption in clinical care. Across clinical settings, the primary barriers are not a lack of innovation, but uncertainty, fragmentation, and limited trust in real-world deployment. AI adoption succeeds when incentives, governance, workflows, payment structures, and data environments are aligned; when they are not, even technically strong tools fail to scale.

MAIC recommends that HHS prioritize a practical adoption agenda across regulation, reimbursement, and research and development:

- Reduce uncertainty with a predictable, risk-tiered approach to AI governance and clearer expectations for pre-deployment testing, post-deployment monitoring, and lifecycle oversight.
- Treat interoperability and data liquidity as foundational infrastructure for clinical AI, using existing authorities to align incentives and reduce fragmentation across data systems.
- Strengthen trust infrastructure as safety infrastructure — including privacy-preserving identity assurance, patient-context integrity, and auditable human–AI attribution mechanisms: privacy-preserving identity assurance, patient-context integrity, attribution of human-AI decision pathways, and auditable interaction records.
- Enable regulatory-safe pilot environments and evidence-generating pathways so health systems can test AI under defined guardrails while producing actionable real-world evidence.
- Modernize reimbursement to reward measurable outcomes, productivity, and reduced clinician burden rather than AI utilization alone, while maintaining guardrails against low-value deployment.
- Invest in applied, implementation-focused research; representative, privacy-preserving datasets; and multi-site validation networks to accelerate shared learning and equitable performance.

These recommendations are grounded in real-world deployment experience and reflect MAIC's role as a multi-stakeholder convening platform connecting clinicians, health system operators,

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technologists, researchers, and community leaders. The goal is not only more capable AI, but more trustworthy and usable AI in real clinical environments.

About the Miami AI Club (MAIC)

The Miami AI Club (MAIC) is a not-for-profit multi-stakeholder convening platform dedicated to advancing responsible, human-centered artificial intelligence across healthcare, education, and public-sector systems. MAIC brings together clinicians, health system executives, researchers, technologists, policymakers, investors, and patient advocates to examine real-world AI deployment challenges and opportunities.

Through its Healthcare Task Force, MAIC focuses specifically on the safe, effective, and accountable integration of AI into clinical care environments. Discussions emphasize implementation science, governance frameworks, interoperability, clinician workflow integration, reimbursement alignment, and public trust.

MAIC does not represent a single vendor or commercial interest. Rather, it serves as a neutral forum for cross-sector dialogue and practical deployment insights. The recommendations in this submission reflect collective observations from real-world AI implementation environments, with particular attention to regulatory clarity, incentive alignment, accountability infrastructure, and patient-centered outcomes.

Cross-Cutting Observations

Across clinical settings, the primary barriers to AI adoption are not a lack of innovation or technical capability, but uncertainty, fragmentation, and limited trust in real-world deployment.

AI adoption in healthcare is multi-pronged. Successful use depends on alignment among incentives, governance structures, clinical workflows, payment structures, and data environments. When these elements are not aligned, even technically strong tools struggle to scale.

Trust remains central. Clinicians and patients must be confident in how AI tools are evaluated, how decisions are attributed, how data are protected, and how human oversight is maintained. Clear expectations and shared frameworks can accelerate adoption more effectively than volume of regulation alone.

Fragmentation across stakeholders, data systems, and evaluation approaches slows collective learning. Policies that encourage interoperability, real-world validation, and coordination can unlock safe and meaningful adoption.

The following responses reflect practical deployment experience and focus on enabling trustworthy, outcome-driven AI in clinical care.

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Regulation

AI is advancing more rapidly than our readiness to deploy it safely and consistently in clinical settings. Numerous applications enter the market daily, yet adoption of AI in clinical care lags far behind its use in administrative functions and other industries. This gap is driven less by lack of innovation and more by lack of regulatory clarity, predictable guardrails, and trusted deployment frameworks.

At present, the national approach to clinical AI governance remains fragmented and unevenly understood. The absence of clear interoperability expectations, evaluation standards, accountability structures, and risk-sharing mechanisms reduces confidence among clinicians and health systems. Addressing these gaps can accelerate adoption while preserving safety and public trust.

As members of the Miami AI Club (MAIC), we regularly convene organizations that are building, buying, evaluating, investing in, and using AI tools in clinical environments. The recommendations below reflect practical, real-world deployment experience:

1. Move Toward a Uniform, Predictable Regulatory Framework

A patchwork of federal guidance and emerging state-level rules creates uncertainty for innovators and implementers. Greater national alignment and clarity would reduce hesitation and encourage responsible investment. A predictable, risk-tiered framework proportionate to clinical impact would allow stakeholders to understand expectations while preserving flexibility for innovation. Clarity often accelerates safe adoption more than additional regulation.

2. Advance Interoperability as Foundational Infrastructure

Clinical data remain fragmented across EHRs, claims, labs, pharmacies, genomics platforms, and consumer sources. AI performance and safety depend heavily on data completeness and context. Encouraging enforceable interoperability standards and aligned incentives would unlock higher-quality AI development and more equitable performance across populations. Interoperability is not only a technical issue but also an incentive and coordination issue.

3. Support Shared Evaluation and Risk Frameworks

Vendors and health systems lack common frameworks for assessing safety, performance, and risk. This creates uncertainty for decision-makers and slows vendor selection and deployment. Existing industry efforts, such as those from the Coalition for Health AI (CHAI) and others, provide useful starting points. HHS could recognize and build upon such work to encourage shared evaluation approaches without mandating a single methodology. Clear expectations for pre-deployment testing, post-deployment monitoring, and lifecycle oversight would foster trust while allowing adaptive AI systems to improve over time.

4. Encourage Development of AI Risk-Sharing and Insurance Models

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AI is probabilistic by nature. Risk can be reduced but not eliminated. Today, many health systems remain cautious because liability and insurance models for AI-related risk are immature. Encouraging dialogue with insurers and exploring incentive structures for AI risk underwriting could reduce adoption barriers. Clearer accountability and risk-sharing frameworks would support responsible deployment.

5. Promote Data Transparency

Bias and uneven performance remain legitimate concerns. Transparency around training and testing data, including demographics, geography, and clinical context, helps clinicians assess applicability and limitations. Emerging 'nutrition label' style disclosures are a promising approach. HHS could encourage transparency norms that support equity and informed decision-making without exposing proprietary information.

6. Strengthen Trust Infrastructure: Identity, Authentication, and Accountability

Regulation often focuses on data and algorithms but less on the infrastructure surrounding AI use. Clinical AI increasingly influences decisions, documentation, and workflows. Yet there are no consistent expectations for verifying clinician identity during AI use; ensuring correct patient-context matching; tracking who accepted or modified AI recommendations; and maintaining reliable audit trails of human-AI interactions. Without strong identity, authentication, and accountability mechanisms, liability frameworks remain difficult to operationalize. Privacy-preserving identity assurance and auditable human-AI interaction records should be viewed as safety infrastructure, not optional features.

7. Enable Regulatory-Safe Pilot Environments

Innovation is slowed by uncertainty around acceptable experimentation. Regulatory-safe pilot environments could allow health systems to test AI tools under defined guardrails, enabling real-world learning while managing risk. Such pilots can surface risks earlier, generate practical evidence, inform future rulemaking, and encourage responsible innovation.

8. Play a Convening Role to Reduce Fragmentation

A major barrier to adoption is fragmentation among stakeholders. Hospitals, developers, researchers, and policymakers often operate in silos, limiting shared learning. HHS is well positioned to serve as a convener to align stakeholders, share deployment lessons, and highlight best practices without adding regulatory burden. Improved coordination can accelerate adoption as much as formal regulation.

Summary: Effective clinical AI regulation should emphasize clarity, predictability, proportional oversight, and trust-building infrastructure. Aligning incentives, supporting interoperability, and encouraging shared learning can accelerate safe adoption while maintaining public confidence.

Reimbursement

HHS payment policies strongly shape how care is delivered and which innovations are adopted. While AI has the potential to enhance productivity, reduce burden, and improve outcomes, current reimbursement structures often do not reward these benefits.

Traditional fee-for-service (FFS) models emphasize volume over outcomes and can inadvertently discourage adoption of technologies that improve efficiency or reduce unnecessary services. As a result, even high-value AI tools may struggle to achieve sustainable adoption. Aligning payment policy with measurable clinical and operational value can accelerate responsible AI use in clinical care.

1. Align Reimbursement With Outcomes and Productivity

AI should not be reimbursed simply for being used. It should be rewarded when it demonstrably improves patient outcomes, reduces avoidable utilization, enhances diagnostic accuracy, enables earlier interventions, reduces clinician administrative burden, or improves care coordination. Outcome-aligned payment and value-based models are better suited to capture AI's benefits than FFS structures.

2. Recognize Clinician Time and Workflow Impact

Many AI tools require clinician supervision, prompting, and validation. These activities carry real opportunity costs. If AI increases clinician workload or cognitive burden, adoption will remain limited regardless of technical performance. Payment models that recognize workflow efficiency and clinician time savings can better reflect the true value of AI-enabled care.

3. Encourage Evidence-Generating Reimbursement Models

AI systems improve over time and may require real-world learning. Traditional reimbursement models often expect mature evidence upfront. HHS could explore coverage-with-evidence-development approaches, pilot payment models, or time-limited reimbursement pathways that allow responsible testing and validation in real-world settings.

4. Reward System-Level Impact, Not Just Tool Adoption

The greatest value from AI often occurs at the system level rather than the individual encounter. Examples include reduced readmissions, improved population health management, earlier disease detection, more efficient resource allocation, better medication adherence, and improved equity and access. Payment models that reward system-level outcomes can better capture AI's value than encounter-based reimbursement.

5. Support Physician-Led Evidence and Clinical Validation

Physician-led evidence and peer-reviewed validation strongly influence adoption. Encouraging clinical leadership in AI evaluation and deployment increases trust and accelerates uptake.

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Payment policies that support evidence generation and dissemination can help scale effective tools.

6. Maintain Guardrails Against Overuse and 'AI for AI's Sake'

Reimbursement policy should avoid incentivizing unnecessary AI deployment or low-value use. Clear expectations for demonstrated value, safety, and equity can prevent waste while supporting meaningful innovation.

Summary: Modernized payment policy can be a powerful lever for responsible AI adoption. Aligning reimbursement with outcomes, productivity, and workflow improvement rather than mere utilization will encourage deployment of AI that benefits patients, clinicians, and the healthcare system.

Research and Development

HHS supports one of the world's largest health research ecosystems and plays a critical role in translating innovation into clinical impact. In AI, the challenge is not only invention but also safe, scalable implementation.

Effective R&D policy should focus on enabling applied research, real-world validation, and shared learning that accelerates responsible adoption across the healthcare system.

1. Prioritize Applied and Implementation-Focused Research

Many AI tools show promise in controlled settings but face challenges in clinical workflows. Greater emphasis on implementation science, workflow integration research, human-AI collaboration models, and deployment in diverse clinical environments can help close the gap between prototype and practice.

2. Invest in High-Quality, Privacy-Preserving Datasets

Standardized, well-governed datasets are foundational for safe and equitable AI. HHS can support creation of representative, high-quality datasets; privacy-preserving data-sharing frameworks; de-identification best practices; mechanisms for patient consent and transparency; and inclusion of diverse populations to reduce bias.

3. Support Real-World Testing and Pilot Networks

Safe real-world testing environments are critical for learning. HHS should encourage multi-site pilot programs, structured real-world evaluations, post-deployment monitoring networks, and cross-institution learning collaboratives.

4. Encourage Public-Private and Multi-Stakeholder Collaboration

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AI progress depends on coordination among health systems, researchers, developers, patient communities, and standards organizations. HHS can act as a facilitator of collaboration without directing specific outcomes.

5. Consider a Lightweight National Coordination Forum

A recurring barrier in U.S. healthcare AI is fragmentation across stakeholders. HHS should consider encouraging a lightweight, multi-stakeholder coordination forum focused on practical deployment and shared learning to share real-world lessons, surface safety and implementation challenges early, identify emerging best practices, and encourage alignment around evaluation approaches.

6. Incorporate International Lessons

Global experience suggests that clear policy direction reduces hesitation; coordinated ecosystems accelerate learning; safe pilot environments enable responsible experimentation; data liquidity supports innovation; and outcome-focused deployment encourages pragmatism.

Summary: R&D policy is most impactful when it supports shared infrastructure, applied research, and real-world validation. By enabling collaboration, data access, and safe pilots, HHS can catalyze innovation without needing to select specific technologies or vendors.

Responses to Specific Questions

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The primary barriers to private sector innovation and adoption of AI in clinical care are not a lack of technical capability, but uncertainty, fragmentation, and limited trust in real-world deployment environments. Key barriers include:

Regulatory Fragmentation and Uncertainty

A patchwork of federal guidance and emerging state-level regulations creates uncertainty for developers and health systems. Predictable, risk-tiered regulatory clarity would reduce hesitation and enable responsible innovation.

Lack of Shared Risk, Evaluation, and Monitoring Standards

There are no commonly accepted frameworks for risk assessment and mitigation, ongoing performance monitoring, real-world validation, and lifecycle oversight of adaptive AI systems. Without shared standards, vendors struggle to demonstrate safety and health systems struggle to evaluate it.

Immature Liability and Insurance Models

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Liability and accountability remain unresolved when AI contributes to clinical decisions, and AI-specific insurance and risk-sharing models are underdeveloped. This leads to understandable risk aversion among providers.

Economic and Workflow Realities in Clinical Settings

For AI to be adopted, it must fit clinical workflow and demonstrate real value. Many tools require clinician supervision, prompting, and validation; if AI increases cognitive or administrative burden, adoption declines.

Data Fragmentation and Interoperability Gaps

Clinical data remain siloed across EHRs, claims systems, labs, pharmacies, and consumer sources. Limited interoperability restricts model development, validation, and equitable performance across populations.

Trust, Identity, and Accountability Infrastructure Gaps

Clinical AI increasingly influences decisions and documentation, yet there are few consistent standards for verifying clinician identity during AI use, ensuring correct patient-context matching, tracking acceptance or modification of AI outputs, and maintaining reliable audit trails.

Privacy and Security Concerns

AI can increase data flows and access points, raising concerns about data misuse, unauthorized access, and centralization risks. Providers and patients need confidence that AI adoption will not increase vulnerability.

Summary: The biggest barriers to AI adoption in clinical care are uncertainty, fragmentation, and trust gaps, not a lack of innovation. Addressing regulatory clarity, interoperability, accountability frameworks, and workflow realities would significantly accelerate responsible adoption.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

To incentivize effective AI use in clinical care, HHS priorities should focus on clarity, proportional oversight, and alignment of incentives with real-world value. The following areas would have the greatest impact:

Regulatory Priorities: Clarity, Risk-Tiering, and Accountability

HHS could prioritize risk-tiered regulatory approaches aligned with clinical impact; clear expectations for pre- and post-deployment monitoring; lifecycle oversight for adaptive AI

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systems; and guidance for non-device clinical AI tools. Existing HIPAA authorities (45 CFR Parts 160 and 164) could be revisited to provide clearer guidance on secondary data use, model training and inference, and data minimization. ONC authorities related to interoperability and information blocking (e.g., 45 CFR Part 171) may also be revisited to strengthen data liquidity for clinical AI while preserving trust.

Strengthen Accountability and Attribution Frameworks

HHS should encourage frameworks that support reliable attribution of human-AI decision pathways, auditability of AI-assisted clinical decisions, and documentation standards for AI-supported care.

Payment Policy Priorities: Reward Outcomes, Not AI Usage

Reimbursement should reward the value AI produces, including measurable improvements in quality, safety, efficiency, and reductions in clinician burden. Expanding outcome-aligned and value-based payment models will better capture AI's benefits than fee-for-service.

Evidence-Generating and Pilot Payment Models

HHS could explore coverage-with-evidence-development approaches, time-limited pilot reimbursement models, and structured pathways for real-world evidence generation.

Programmatic Design: Support Interoperability and Data Liquidity

HHS could prioritize enforceable interoperability standards, alignment of incentives for data sharing, guidance on de-identification and privacy-preserving data use, and encouraging representative datasets to reduce bias.

Support Safe Innovation Environments

Programmatic support for multi-site pilots, structured real-world evaluations, and cross-institution learning networks would allow innovation and safety to evolve together.

Summary: The most impactful changes are those that reduce uncertainty, align incentives with outcomes, and support real-world learning. Clear guidance, proportional oversight, and modernized payment models can accelerate responsible AI adoption while maintaining safety and trust.

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3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

The use of AI in clinical care introduces legal and implementation questions that existing governance frameworks were not designed to fully address. These issues benefit from clearer guidance and updated interpretation in AI-specific contexts. Key issues include:

Attribution and Liability in AI-Assisted Decisions

Responsibility can become ambiguous when AI contributes to clinical decision-making. Clear expectations for documentation, human oversight, and attribution can help clarify accountability without removing clinician judgment.

Governance of Adaptive and Continuously Learning Systems

Continuously learning systems raise questions about when revalidation is required, how drift is monitored, and how updates are governed and documented.

Transparency and Representativeness of Training and Testing Data

Performance varies across populations and settings. Encouraging transparent disclosure of data characteristics and limitations supports informed use and reduces unintended disparities.

Movement of Data Between HIPAA and Non-HIPAA Contexts

AI can involve data flows between covered entities and consumer technologies, creating ambiguity around secondary use and responsibilities for safeguarding data.

Identity, Authentication, and Auditability

Implementation requires reliable attribution of actions, correct patient context, and audit trails of human-AI interactions. HHS can encourage standards that strengthen attribution and auditability.

Misrepresentation of AI Capabilities

Overstating AI performance can create legal and safety risks; evidence-based claims and transparency around limitations protect patients and providers.

Summary: Most legal and implementation challenges stem from ambiguity rather than absence of law. Clearer guidance on attribution, data governance, lifecycle oversight, and transparency can enable safe adoption while maintaining accountability and public trust.

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4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Effective evaluation of clinical AI requires both technical validation and real-world assessment. Model performance alone is insufficient; evaluation must reflect how AI functions in clinical environments and how humans interact with it. Promising approaches include:

Risk-Based Evaluation Frameworks

Evaluation should be proportional to clinical impact and potential harm, incorporating patient safety impact, consequences of failure, population vulnerability, environment of use, and equity considerations. HHS can encourage convergence around shared principles without mandating a single framework.

Human-in-the-Loop and Workflow Evaluation

Evaluation should assess cognitive load, alert fatigue, usability and workflow integration, supervision burden, and ability for timely human intervention.

Real-World Performance Monitoring

Ongoing evaluation should include post-deployment monitoring, drift detection, periodic revalidation, and performance across diverse populations and settings.

Transparency in Training and Testing Data

Evaluation should consider data representativeness, demographic coverage, geographic diversity, and clinical context of testing.

Auditability and Traceability

Frameworks should include the ability to trace how outputs were generated, review decision pathways, and maintain records of human-AI interaction.

HHS Support: Yes. Impactful mechanisms include targeted grants for evaluation research, cooperative agreements supporting multi-site validation, prize challenges encouraging robust testing methods, and support for post-deployment learning networks.

Summary: The most effective evaluation approaches combine technical performance, human factors, and real-world monitoring. Supporting shared evaluation principles and real-world learning can accelerate safe adoption while maintaining flexibility for innovation.

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5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS can enable private sector validation and accountability while avoiding overly prescriptive certification structures that could slow innovation. The most effective support would focus on clarity, alignment, and shared learning rather than centralized control:

Encourage Voluntary, Industry-Led Standards

Recognize credible industry-led frameworks, encourage transparency and comparability, and highlight best practices.

Support Independent Real-World Evaluation

Encourage third-party testing and benchmarking, multi-site validation studies, and real-world performance reporting.

Promote Transparency and Disclosure Norms

Encourage standardized disclosures around intended use, data representativeness, known limitations, and appropriate supervision levels.

Integrate AI Competencies Into Clinical Training

Encourage AI literacy and competency development among clinicians and administrators to improve safe use and informed procurement.

Reduce Structural Barriers to Responsible Innovation

Promote interoperability, clarify data governance expectations, support risk-tiered oversight, and encourage evidence generation.

Summary: The most effective role for HHS is to enable trust, transparency, and shared learning while allowing market-driven validation to evolve. Light-touch support for independent evaluation and disclosure norms can promote safe innovation without unnecessary regulatory burden.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

AI has demonstrated meaningful value in several areas of clinical care, particularly where tasks are data-rich, pattern-based, and augment human expertise. At the same time, some applications have fallen short when expectations exceeded current technical or workflow realities.

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Areas Where AI Has Met or Exceeded Expectations

- Imaging and early detection (radiology, pathology, ophthalmology) where AI augments clinician interpretation.
- Precision medicine and oncology, particularly for navigating complex genomic and clinical data in multidisciplinary settings.
- Drug discovery and clinical research, accelerating early-stage target identification and trial design.
- Population health and risk stratification, supporting preventive interventions and resource allocation.

Areas Where AI Has Fallen Short

- Workflow misalignment: tools that disrupt routines or increase documentation burden.
- Overgeneralization across populations: models that do not generalize well beyond training settings.
- Unrealistic expectations: marketing claims outpacing evidence.
- Insufficient real-world validation: performance drops in variable real-world environments.

Novel AI Tools With High Potential

- Clinician burden reduction tools (documentation, chart review, administrative automation).
- Early disease detection and preventive care tools.
- Care coordination and decision support that synthesizes data across sources.
- Equity-focused AI that identifies disparities and targets interventions.

Summary: AI shows the greatest value when it augments clinicians, fits workflows, and is validated in real-world settings. Its potential to improve outcomes and reduce costs is substantial, but success depends on thoughtful implementation, transparency, and realistic expectations.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Adoption of AI in clinical care is shaped by a combination of executive leadership, clinical leadership, and operational governance. Successful implementation typically requires alignment across these groups rather than reliance on a single decision-maker.

Decision Makers With the Greatest Influence

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- Executive leadership (CEO/COO/CFO) for strategy, financial priorities, and scaling.
- Clinical leadership (CMO, department chairs, physician champions) for trust and clinical validation.
- IT and digital leadership (CIO/CMIO/CDO) for integration, cybersecurity, and data governance.
- Compliance and legal teams for privacy, security, and regulatory considerations.
- AI/digital governance committees for evaluation, risk management, and prioritization.

Primary Administrative Hurdles

- Unclear value proposition and difficulty separating strategic AI from experimentation.
- Limited clinician engagement in evaluation and design.
- Workflow disruption without clear benefit.
- Lack of change management, training, and realistic expectations.
- Uncertainty around internal governance and lifecycle oversight.
- Data quality and integration challenges.

Summary: AI adoption is most successful when it is clinically relevant, strategically aligned, and supported by cross-functional governance. Addressing administrative and organizational realities is as important as technical performance.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability accelerates safe and effective AI development by improving data completeness, context, and representativeness. AI systems perform best when they can learn from and operate across diverse, longitudinal data sources rather than isolated datasets.

Longitudinal, Cross-Setting Clinical Data

Interoperability between EHR systems, claims data, pharmacy systems, laboratory systems, imaging systems, and post-acute and home care records supports better prediction, care coordination, and preventive care.

Integration of Clinical and Social Context

Combining clinical data with social determinants of health, behavioral data, and community-level indicators can improve risk stratification and targeted interventions, with strong governance and transparency.

Standardized, Structured Clinical Data

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Wider adoption of standards such as FHIR, standardized terminologies, and consistent metadata practices improves data usability and comparability.

De-Identification and Privacy-Preserving Linkage

Guidance and tools for reliable de-identification, privacy-preserving record linkage, and patient-consented data use support research while protecting privacy.

Benchmarking and Evaluation Datasets

Shared benchmarking datasets and evaluation frameworks enable fair comparison and transparency; multi-institution datasets reflecting diverse populations reduce bias and improve generalizability.

Interoperability as Incentive Alignment

Interoperability is also an incentive and governance issue. Aligning incentives to support data sharing and standardization can accelerate progress more effectively than technical mandates alone.

Summary: Interoperability widens opportunities when it enables longitudinal, contextual, and representative data use while preserving privacy. Progress depends on standards adoption, incentive alignment, and trusted governance frameworks.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers generally care less about AI itself and more about whether care becomes more timely, accurate, affordable, and humane. AI is viewed as beneficial when it clearly improves care delivery and concerning when it feels opaque or depersonalizing.

Challenges Patients and Caregivers Want Addressed

- ☐ Earlier and more accurate diagnosis.
- ☐ Personalized and coordinated care across providers and settings.
- ☐ Reduced administrative burden and access barriers (wait times, navigation, scheduling).
- ☐ Preventive and proactive care that supports early intervention.
- ☐ Health equity: systems that work across diverse populations without reinforcing disparities.

Patient and Caregiver Concerns

- ☐ Trust and transparency about when and how AI is used.

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- Privacy and data security, especially in light of high-profile breaches.
- Loss of human connection: AI should support, not replace, clinician empathy and judgment.
- Bias and fairness across demographic groups.
- Accountability if an AI-supported decision is wrong.
- Assurance of human oversight and ability to override AI recommendations.

Summary: Patients and caregivers support AI when it improves access, accuracy, and coordination while preserving privacy, human oversight, and transparency. Trust grows when AI supports, not replaces, clinician judgment.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care? (a) Are there published findings about the impact of adopted AI tools and their use in clinical care? (b) How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

To accelerate safe and effective adoption, research priorities should emphasize real-world impact, human-AI collaboration, and system-level value rather than model performance alone.

Priority Areas for AI Research

- Implementation and workflow research (human-AI collaboration, cognitive load, supervision models, adoption).
- Real-world effectiveness and generalizability (multi-site validation, diverse populations, longitudinal monitoring, drift and recalibration).
- Equity and bias mitigation (representative datasets, fairness methods, disparity reduction).
- Privacy-preserving and secure AI (federated learning, privacy-preserving analytics, secure data sharing).
- AI for administrative burden reduction (documentation and workload reduction).

Published Findings on AI in Clinical Care

The literature shows promising results in imaging and diagnostics, oncology and genomics, risk prediction and population health, and clinical decision support. However, many studies are retrospective or limited in scale; more prospective real-world evidence is needed.

Costs, Benefits, and Transfers

Literature suggests AI can improve efficiency and support earlier intervention, but integration costs can be significant. Benefits often accrue at system or population levels rather than individual

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encounters, and value depends heavily on workflow integration and governance. Organizational readiness and implementation quality strongly influence ROI.

Summary: Research that links technical performance to real-world clinical value, safety, and equity will most effectively accelerate adoption. Prioritizing implementation science and real-world validation can help translate AI's promise into measurable health outcomes.

Closing

MAIC appreciates the opportunity to provide input. The policy levers most likely to accelerate safe adoption are those that reduce uncertainty, strengthen interoperability and trust infrastructure, align incentives with outcomes, and enable real-world learning. We welcome continued engagement to support shared learning and responsible deployment. As AI adoption accelerates, establishing durable trust infrastructure will be as important as advancing model capability.

Through its Healthcare Task Force, MAIC is continuing structured dialogue and implementation-focused collaboration to translate policy guidance into practical adoption frameworks. We welcome continued engagement with HHS and other stakeholders to support safe, accountable, and scalable AI deployment.

Submitted By

Miami AI Club (MAIC) Healthcare Task Force: Contributors include clinical leaders, healthcare system strategists, and technology governance advisors with experience deploying AI in real-world care environments.

Task Force Leadership

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